

CITYVIEW MEDICAL CENTER

CLINICAL HISTORY AND RECORD

MRN: 2024-05-001234
Encounter ID: ENC-2024-050678
Date of Visit: May 12, 2024

1. PATIENT IDENTIFICATION

Full Name: John Michael Doe
Date of Birth: April 23, 1978 (46 y/o)
Gender: Male
Address: 1254 Oak Street, Apt 5B
Springfield, IL 62701
Phone: (217) 555-0198
Email: johndoe@email.com
Marital Status: Married
Occupation: Software Engineer
Primary Language: English
Insurance: HealthPlus PPO
Policy No.: HP-77889900

2. CHIEF COMPLAINT

Persistent cough and shortness of breath for 1 week.

3. HISTORY OF PRESENT ILLNESS

The patient is a 46-year-old male who presents with a 1-week history of productive cough with yellowish sputum, associated with shortness of breath on exertion and mild fever. Symptoms started gradually and have been worsening over the past 3 days.

He denies chest pain, hemoptysis, or recent travel. No known contact with sick individuals.

He took over-the-counter cough medication with minimal relief.

4. PAST MEDICAL HISTORY

- Hypertension (diagnosed in 2021)
- Dyslipidemia
- No history of diabetes, asthma, or chronic lung disease
- No prior surgeries

5. MEDICATIONS

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Medication	Dose	Route	Frequency	Indication
Lisinopril	10 mg	PO	Once daily	Hypertension
Atorvastatin	20 mg	PO	Once daily (at night)	Dyslipidemia
Acetaminophen	500 mg	PO	PRN	Fever / Pain

6. ALLERGIES

No known drug allergies.

7. FAMILY HISTORY

Father: Hypertension, Type 2 Diabetes Mellitus

Mother: Hyperlipidemia

No family history of cancer or heart disease.

8. SOCIAL HISTORY

Smoking: Former smoker, 10 pack-years, quit in 2018
Illicit Drugs: Denies

Alcohol: Social drinker (1–2 drinks/week)
Exercise: Moderate (2–3 times/week)

Diet: Regular diet

Living Sit: Lives with wife and 2 children

9. REVIEW OF SYSTEMS

General: Fever, fatigue. No weight loss.
Cardio: No chest pain, no palpitations.

HEENT: No headache, no sore throat.
Respiratory: Cough, shortness of breath.

Gastro: No nausea, no vomiting.
Urinary: No dysuria, no frequency.

10. PHYSICAL EXAMINATION

Vital Signs

Temperature	38.2 °C
Blood Pressure	128/76 mmHg
Heart Rate	98 bpm
Respiratory Rate	20 breaths/min
Oxygen Saturation	94% on room air

General: Alert, oriented, in no acute distress.
HEENT: No pallor, no icterus, oropharynx clear.
Neck: Supple, no lymphadenopathy.
Cardiovascular: S1 and S2 normal, no murmurs.
Respiratory: Crackles auscultated over the right lower lung field.
Abdomen: Soft, non-tender, no organomegaly.
Extremities: No edema, peripheral pulses normal.
Neurological: No focal deficits.

11. IMAGING

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Chest X-Ray (PA View) – May 12, 2024

Findings: Patchy infiltrate in the right lower lung field. No pleural effusion. Cardiomeastinal silhouette within normal limits.

12. LABORATORY RESULTS

Test	Result	Reference Range	Units
WBC	12.5	4.0 – 11.0	x103/uL
Hemoglobin	14.2	13.5 – 17.5	g/dL
Hematocrit	42.1	41 – 53	%
Platelets	280	150 – 450	x103/uL
Neutrophils	78	40 – 70	%
Lymphocytes	14	20 – 40	%
C-Reactive Protein (CRP)	25.6	< 5.0	mg/L
Sodium	139	135 – 145	mmol/L
Potassium	4.1	3.5 – 5.0	mmol/L
Creatinine	0.94	0.60 – 1.20	mg/L

13. ASSESSMENT

- Community-acquired pneumonia (CAP), right lower lobe.
- Hypertension, well-controlled.
- Dyslipidemia.

14. PLAN

- Start antibiotics: Amoxicillin/Clavulanate 875/125 mg PO every 12 hours for 7 days.
- Supportive care: Increase fluid intake, rest, acetaminophen for fever as needed.
- Continue maintenance medications: Lisinopril and Atorvastatin.
- Follow-up in 5–7 days or earlier if symptoms worsen.
- Repeat chest X-ray in 4 weeks to ensure resolution.

